Waqas Rabbani

SPMM COURSE

Started on	Sunday, 24 July 2022, 4:16 PM
State	Finished
Completed on	Sunday, 24 July 2022, 4:16 PM
Time taken	6 secs
Marks	0.00/200.00
Grade	**0.00** out of 100.00

Question **1**

Not answered

Marked out of 4.00

Dysarthria

Choose one aetiology for each of the following clinical situations;

Mrs. W is a 55-year-old woman who presents with slow scanning speech. She speaks with varying force and appears to be broken down into syllables. Choose...

Mr. X is a 63-year-old man who presents with slow, slurred and monotonous speech. The pitch is high. On examination, his tongue is small and contracted. Choose...

Mr. Y is a 69-year-old man who presents with a speech of nasal quality. It is slurred and indistinct. Choose...

Miss. Z is a 45-year-old woman with sudden loss of speech following an argument with her boyfriend Choose...

Choose...
Upper motor neuron lesions
Laryngeal lesions
Lesions of Broca's area
Cerebellar lesions
Vocal cord paralysis
Lower motor neuron lesions
Extrapyramidal lesions
Lesion's of Wernicke's area
Conversion disorder

In cerebellar lesion scanning or ataxic speech is seen. Intonation, pitch and volume and also be affected, as well as difficulty with alternate tongue movements.

Flaccid dysarthria can be of various qualities depending on which motor neuron is affected in a LMN type of lesion. The tongue shows loss of tone if XII nerve is involved and lies flaccidly on the floor of the mouth. In LMN lesions, speech could be of nasal quality if IX and X nerves are involved.

In UMN lesions (e.g. pseudobulbar palsy), small and contracted tongue accompanies a high pitched speech tone.

In hysterical aphonia, the examination is usually normal. Sudden loss of voice following a stressor, but preserved vocal cord activity is notable.

The correct answer is: Mrs. W is a 55-year-old woman who presents with slow scanning speech. She speaks with varying force and appears to be broken down into syllables. → Cerebellar lesions, Mr. X is a 63-year-old man who presents with slow, slurred and monotonous speech. The pitch is high. On examination, his tongue is small and contracted. → Upper motor neuron lesions, Mr. Y is a 69-year-old man who presents with a speech of nasal quality. It is slurred and indistinct. → Lower motor neuron lesions, Miss. Z is a 45-year-old woman with sudden loss of speech following an argument with her boyfriend → Conversion disorder

Question **2**

Not answered

Schizophrenic speech disturbance

For each description given below choose the most appropriate term from the list provided:

Marked out of 4.00

A disorder in the logical progression of thoughts in which unrelated and unconnected ideas shift from one subject to another. Choose...

A pattern of speech, which is indirect and delayed in reaching its goal idea. Choose...

Words are chosen for common sounds of initial syllabi rather than the meaning. Choose...

Making up a totally new word that is not in the dictionary. Choose...

Choose...
Flight of Ideas
Clanging
Circumstantiality
Echolalia
Neologisms
Tangentiality
Derailment
Logoclonia
Thought blocking
Perseveration

Derailment is a symptom of schizophrenic thought disorder in which there is a total break in the chain of association among succeeding thoughts. The relationship between the two sequential ideas expressed in the speech is unclear to both the patient and the examiner. In **circumstantiality**, thinking proceeds slowly, with many unnecessary details and digressions, before returning to the point. It is seen in some patients with temporal lobe epilepsy or alcohol-induced persisting dementia, learning difficulty and in obsessional personalities. The verbal associations (due to words spoken) are called **clang associations** where thoughts are associated with the structure of words rather than their meaning. In schizophrenic FTD, clanging occurs more often with the first syllables as opposed to clangs seen in poetry, humour and manic speech where they occur more at the end syllables. **Neologisms** refers to making up a totally new word that is not in dictionary or using a known word with an entirely different meaning e.g. 'Inkur' for pen (new) or 'roast' for pen (different).

The correct answer is: A disorder in the logical progression of thoughts in which unrelated and unconnected ideas shift from one subject to another. → Derailment, A pattern of speech, which is indirect and delayed in reaching its goal idea. → Circumstantiality, Words are chosen for common sounds of initial syllabi rather than the meaning. → Clanging, Making up a totally new word that is not in the dictionary. → Neologisms

Question **3**

Not answered

Marked out of 3.00

Differential diagnoses

Choose the most likely diagnosis for each of the following situations;

A 40-year-old woman was robbed in an underground station five years ago. Since then, she is fearful to leave her flat. When she goes to supermarkets, she freezes and feels that she might even die. Choose...

A 23-year-old woman works in a warehouse. She is very anxious to speak even to her colleagues and boss. She does not socialise, never been to a pub and isolates herself. She is worried about causing embarrassment when talking in front of others Choose...

A 26-year-old woman lost her dad in a car accident three weeks ago. She has not taken the car since this incident and doesn't want to use the same route. She feels as if her dad is still with her. Choose...

Choose...
Bipolar disorder
Adjustment disorder
Conversion disorder
Agoraphobia with panic disorder
Social phobia
Schizophrenia
Post-traumatic stress disorder
Depression
Normal grief reaction

Case 1 describes panic disorder and agoraphobia. Case 2 is a clear example of social phobia. In question 3, normal grief - not post-traumatic stress - is described. Unlike depression, normal but intense grief is characterized by the absence of shame or significant guilt. According to Ng (2005), "guilt, if it exists, is that of omission (what the bereaved person may have failed to do) rather than

commission (the recognition of malevolent intent)". Grief often fluctuates, and, even in intense grief, distraction is often possible, and pleasant, happy memories are intermixed with more painful ones. Although life may seem empty, there is often a cognitive awareness of the experience as a time-limited process. Suicidal thoughts are passive when they exist; less intense and suicidal acts are rare in the grieving person.

(Ref: Am J Psychiatry 152:1, January 1995 and http://www.ncbi.nlm.nih.gov/pubmed/16021224).

The correct answer is: A 40-year-old woman was robbed in an underground station five years ago. Since then, she is fearful to leave her flat. When she goes to supermarkets, she freezes and feels that she might even die. → Agoraphobia with panic disorder, A 23-year-old woman works in a warehouse. She is very anxious to speak even to her colleagues and boss. She does not socialise, never been to a pub and isolates herself. She is worried about causing embarrassment when talking in front of others → Social phobia, A 26-year-old woman lost her dad in a car accident three weeks ago. She has not taken the car since this incident and doesn't want to use the same route. She feels as if her dad is still with her. → Normal grief reaction

Question **4**

Not answered

Marked out of 4.00

Rating scales - Properties

Which of the listed items describes the attributes of the scale that you are testing?

You ask patients to complete an anxiety rating scale six months apart — Choose...

You look back at all your patients whom you have treated for depression to see if their baseline rating scales are related to their current mental state — Choose...

You asked a set of nurses, doctors and former patients to give their opinion as to whether you are asking the appropriate questions about levels of satisfaction on the ward — Choose...

You are administering a putative test of intelligence and WAIS (Weschler adult intelligence scale) at the same time, and the correlation between the two is computed — Choose...

Choose...
Predictive validity
Interrater reliability
Criterion validity
Internal consistency reliability
Concussion validity
Content validity
Ecological validity
Test re-test reliability
Concurrent validity

Explanation:

Test-retest reliability is a measure of a test's reliability, or more specifically its stability, based on the correlation between scores of a group of respondents on two separate occasions.

In predictive validity, the predictor scores (in this case, baseline rating scales) are obtained in advan of the criterion scores (current mental state). It is the ability of a test to predict future group differences according to current group differences.

Content validity is the extent to which items of a test are appropriate to the content domain of the test. For example, a test used for employment selection has high content validity if the test items cover the abilities, skills, or competencies required to perform the job and do not cover other irrelevant attributes.

To determine test concurrent validity, external measures (in this case, the WAIS) are obtained at the same time that the test is given to the sample of subjects.

Ref: Colman AM. Oxford Dictionary of Psychology, 4 th Edition. 2015. pp. 164, 593, 761.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 57.

The correct answer is: You ask patients to complete an anxiety rating scale six months apart → Test re-test reliability, You look back at all your patients whom you have treated for depression to see if their baseline rating scales are related to their current mental state → Predictive validity, You asked a

set of nurses, doctors and former patients to give their opinion as to whether you are asking the appropriate questions about levels of satisfaction on the ward → Content validity, You are administering a putative test of intelligence and WAIS (Weschler adult intelligence scale) at the same time, and the correlation between the two is computed → Concurrent validity

Question **5**

Not answered

Marked out of 4.00

Sleep disturbances

Choose one option for each of the following descriptions

This is a type of sleep disorder featured by stereotyped, repetitive movements usually involving the head and neck. It occurs immediately prior to sleep which are sustained into light sleep. Choose...

This is a type of sleep disorder in which the individuals have more than 24 hour sleep-wake period, which leads to a steady pattern of 1-2 hour daily delays in sleep onset and wake times. Choose...

Mr. A's partner is concerned that he presents with sudden abrupt contractions of muscle groups mainly in his legs but sometimes involves the arms, which occurs mainly at sleep onset. Choose...

Mr. A is complaining of evening sleepiness, early sleep onset by 7 pm and early morning wakening. Choose...

Choose...
Advanced sleep-wake phase disorder
Sleep apnoea
Circadian rhythm disorder-not otherwise specified
Rhythmic movement disorder
Hypnic jerks
Kleine-Levin syndrome
Narcolepsy syndrome
Non 24 hour sleep wake disorder
Recurrent hypersomnia
Shift work sleep disorder

Explanation:

Sleep-related rhythmic movement disorder is characterised by stereotyped repetitive movements involving large muscles, usually the head and neck (may lead to head injury), typically immediately prior to sleep, sustained into light sleep.

Non-24-hour sleep-wake rhythm disorder is characterised by the rare occurrence of >24-hour sleep-wake period (also called 'free-running' or 'non-entrained'), leading to a chronic pattern of 1-2 hour daily delays in sleep onset and wake times. It is common in totally blind individuals. In non-blind patients, some environmental conditions can lead to its appearance (insufficient or time-inappropriate exposure to circadian-entraining agents such as light).

Hypnic jerks (or 'sleep starts') occur at sleep onset and present as sudden, abrupt contractions of muscles groups, usually the legs, but sometimes also involving the arms, neck, or even the entire body.

Advanced sleep-wake phase disorder leads to complaints to evening sleepiness, early sleep onset (e.g. 18.00-20.00), and early morning wakening.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 454-455, 476-477.

The correct answer is: This is a type of sleep disorder featured by stereotyped, repetitive movements usually involving the head and neck. It occurs immediately prior to sleep which are sustained into light sleep. → Rhythmic movement disorder, This is a type of sleep disorder in which the individuals have more than 24 hour sleep-wake period, which leads to a steady pattern of 1-2 hour daily delays in sleep onset and wake times. → Non 24 hour sleep wake disorder, Mr. A's partner is concerned that he

presents with sudden abrupt contractions of muscle groups mainly in his legs but sometimes involves the arms, which occurs mainly at sleep onset. → Hypnic jerks, Mr. A is complaining of evening sleepiness, early sleep onset by 7 pm and early morning wakening. → Advanced sleep-wake phase disorder

Question **6**

Not answered

Marked out of 4.00

Motor symptoms of schizophrenia

Identify the terms used to describe the following presentations;

While examining a 45-year-old man with schizophrenia, the psychiatrist places the patient's arms in a folded position which the patient then holds in the same position for a long period. Choose...

While examining a 36-year-old man, he resists passive movement with same degree of force as applied by the examiner. Choose...

While examining a 30-year-old man with schizophrenia, his limbs may be moved with minimal pressure. Choose...

A 43-year-old man in a rehabilitation unit was observed to be saluting passers-by all the time. Choose...

Choose...
Cataplexy
Automatic obedience
Mitgehen
Mannerism
Psychological pillow
Stereotypy
Gegenhalten
Waxy flexibility

Explanation:

Waxy flexibility (also known as catalepsy or flexibilitas cerea) is a rare motor symptom that describes a situation in which the patient's limbs can be passively moved to any posture, which will then be held for a prolonged period of time.

Gegenhalten is when an individual with catatonia opposes all passive movements with the same degree of force as that which is being applied by the examiner.

Mitmachen is a motor symptom of schizophrenia where the patient's limbs can be moved without resistance to any position. The limbs return to their resting state once the examiner lets go, in contrast with catalepsy where the limbs remain in their set positions for prolonged periods. Mitgehen is an extreme form of mitmachen where the patient's limbs can be moved to any position by very slight or fingertip pressure.

A mannerism is the abnormal and occasionally bizarre performance of a voluntary, goal-directed activity (e.g. a conspicuously dramatic manner of walking).

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 104, 114.

Casey P, Kelly B. Fish's clinical pathology: Signs and symptoms in psychiatry, 4th Edition. 2019. p. 94.

The correct answer is: While examining a 45-year-old man with schizophrenia, the psychiatrist places the patient's arms in a folded position which the patient then holds in the same position for a long period. → Waxy flexibility, While examining a 36-year-old man, he resists passive movement with same degree of force as applied by the examiner. → Gegenhalten, While examining a 30-year-old man with schizophrenia, his limbs may be moved with minimal pressure. → Mitgehen, A 43-year-old man in a rehabilitation unit was observed to be saluting passers-by all the time. → Mannerism

Question **7**

Not answered

Marked out of

Clinical features and psychopathology

Identify the type of psychopathology in each of the following situations

5.00

Choose...
Delusions of reference
Delusional mood
No delusions
Delusions of grandiosity
Fregoli delusion
Nihilistic delusions
Capgras delusion
Delusions of infidelity

A 46-year-old woman has been admitted to the psychiatric ward. This is her first admission. She has low self-confidence and is reported to be withdrawn. She worries that she has no money. She states that she wouldn't be surprised if her boyfriend is having an affair at all, as she is not attractive. Choose...

A 42-year-old man was admitted with a relapse of his psychotic illness. He says that there are hidden television cameras in his room, and he has seen the full interview being broadcast on the television. Choose...

A 45-year-old man with chronic schizophrenia believes that his neighbour can take on different guises. He believes that his neighbour has played a crucial role in disseminating this information on the television. Choose...

Explanation:

Most of the beliefs expressed by patient 1 may be true and are not expressed with absolute conviction and distress. They are unlikely to be delusions.

A delusion of reference is a delusional belief that external events or situations have been arranged in such a way as to have particular significance for, or to convey a message to, the affected individual. The patient may believe that television news items are referring to them or that parts of the Bible are about them directly.

Fregoli syndrome is a type of delusional misidentification in which the patient believes that strangers have been replaced with familiar people. For example, a patient may believe that a neighbour can change his appearance, clothes, and even his sex at will so as to spy on the patient.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 107, 111.

Casey P, Kelly B. Fish's clinical pathology: Signs and symptoms in psychiatry, 4th Edition. 2019. p. 127.

The correct answer is: A 46-year-old woman has been admitted to the psychiatric ward. This is her first admission. She has low self-confidence and is reported to be withdrawn. She worries that she has no money. She states that she wouldn't be surprised if her boyfriend is having an affair at all, as she is not attractive. → No delusions, A 42-year-old man was admitted with a relapse of his psychotic illness. He says that there are hidden television cameras in his room, and he has seen the full interview being broadcast on the television. → Delusions of reference, A 45-year-old man with chronic schizophrenia believes that his neighbour can take on different guises. He believes that his neighbour has played a crucial role in disseminating this information on the television. → Fregoli delusion

Question **8**

Not answered

Marked out of 4.00

Research measures in psychiatry

For each of the following descriptions choose the appropriate instrument from the list provided.

Choose...
Schedule for Assessment in Neuropsychiatry
Diagnostic interview schedule
MADRS
SF-12
CAGE
Clinical Interview Schedule
GHQ-12
MMSE
Geriatric Mental Score

A scale used as a measure of psychiatric caseness in research studies Choose...

A scale used as a measure of depression in research studies Choose...

A scale used as a screening instrument for alcoholism Choose...

A scale used as fully structured, non-

clinician administered tool for making lifetime diagnoses in epidemiological studies — Choose...

Explanation:

The General Health Questionnaire (GHQ) is a self-rated questionnaire used as a screening instrument for the presence of psychiatric illness. The patient is asked to report the presence of a list of symptoms in the preceding weeks. Four versions are available, using 12, 28 (max score = 84), 30, and 60 items.

The Montgomery-Asberg Depression Rating Scale (MADRaS) is a 10-item observer-rated scale of depression. Each item is rated 0-6, with a total score obtained.

The **CAGE** questionnaire is a brief screening tool used for identification of at-risk drinking. It consists of 4 questions**:** Have you ever felt you should **C**ut back on your drinking? Has anyone ever **A**nnoyed you by criticizing your drinking? Have you ever felt **G**uilty about your drinking? Have you ever had a drink early in the morning as an **E**ye-opener?

The Diagnostic Interview Schedule can be used by non-clinicians to administer a fully structured interview, to diagnose the major psychiatric illnesses for research purposes.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 96-97, 582.

The correct answer is: A scale used as a measure of psychiatric caseness in research studies → GHQ-12, A scale used as a measure of depression in research studies → MADRS, A scale used as a screening instrument for alcoholism → CAGE, A scale used as fully structured, non-clinician administered tool for making lifetime diagnoses in epidemiological studies → Diagnostic interview schedule

Question **9**

Not answered

Marked out of 3.00

Scenarios related to psychodynamic defences

Choose the best option from the list provided for each description below.

A 23-year-old woman studying at a university in Newcastle finds herself in London without any recollection of how she got there. — Choose...

A 24 yr old woman waiting for mediastinoscopy has read everything about the procedure and its evidence. She is not concerned about the pain and impact of diagnosis on her emotional state. — Choose...

Soon after the death of her husband, a 48-year-old woman suddenly starts becoming quite flirtatious. She shows inappropriate behaviour towards her dead husband's friends and appears to have had no grief or mourning. On confrontation, she could not explain her behaviour. — Choose...

Choose...
Intellectualization
Passive-aggressive
Isolation
Projection
Dissociation
Acting out
Displacement
Introjection
Rationalization
Denial

Explanation:

Dissociation is a temporary but drastic modification of character or sense of personal identity to avoid emotional distress. It includes fugue states and conversion reactions.

Intellectualisation is characterised by focusing on abstract concepts, logic, and other forms of intellectual reasoning to avoid facing painful emotions (e.g. a victim of a traumatic abuse experience may discuss the statistics of abuse instead of talking about the particular experience).

In acting out, patients directly express unconscious wishes or conflicts through action to avoid being conscious of either the accompanying idea or the affect. Tantrums, apparently motiveless assaults, and pleasureless promiscuity are typical examples.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 577, 1067.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 893.

The correct answer is: A 23-year-old woman studying at a university in Newcastle finds herself in London without any recollection of how she got there. → Dissociation, A 24 yr old woman waiting for mediastinoscopy has read everything about the procedure and its evidence. She is not concerned about the pain and impact of diagnosis on her emotional state. → Intellectualization, Soon after the death of her husband, a 48-year-old woman suddenly starts becoming quite flirtatious. She shows inappropriate behaviour towards her dead husband's friends and appears to have had no grief or mourning. On confrontation, she could not explain her behaviour. → Acting out

Question **10**

Not answered

Marked out of 3.00

Psychodynamic defences

Choose the best option from the list for each description below.

A 42-year-old man has been diagnosed with poor prognostic variant carcinoma of the lung. After hearing the bad news, the patient proceeds to tell his physician how excited he is about attending his son's wedding anniversary two months later. — Choose...

A mother, whose son was killed in a road rage, quits her job and joins a voluntary organization working towards safer driving. — Choose...

After having an argument with his wife, Mark gets into an altercation and fight at the local pub, quite uncharacteristic of him. — Choose...

Choose...
Passive-aggressive
Isolation
Altruism
Rationalization
Acting out
Projection
Displacement
Dissociation
Denial
Intellectualization

Explanation:

Denial is defined as the expressed refusal to recognise a threatening reality. It is of relevance especially to those with serious physical illnesses, where the patient denies being told of the presence of illness in themselves or their loved ones. It may persist despite constant reiteration of the facts.

Altruism describes the mechanism of satisfying one's own needs through the lives of others. It involves vicarious but constructive and instinctually gratifying service to others, even to the detriment of the self.

Displacement is the process by which interest and/or emotion is shifted from one object onto another less threatening one. For example, the person who is having problems at work may displace the anger felt for their boss onto their family by displaying irritability and moodiness at home.

Ref: Casey P, Kelly B. Fish's clinical pathology: Signs and symptoms in psychiatry, 4th Edition. 201 p. 129.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 1067.

The correct answer is: A 42-year-old man has been diagnosed with poor prognostic variant carcinoma of the lung. After hearing the bad news, the patient proceeds to tell his physician how excited he is about attending his son's wedding anniversary two months later. → Denial, A mother, whose son was killed in a road rage, quits her job and joins a voluntary organization working towards safer driving. → Altruism, After having an argument with his wife, Mark gets into an altercation and fight at the local pub, quite uncharacteristic of him. → Displacement

Question **11**

Not answered

The Language of Psychopathology

Identify the terms commonly used in psychopathology using the definitions given in the list.

Marked out of 5.00

Choose...
Agitation
Affect
Mood
Anhedonia
Anxiety
Euphoria
Blunted affect
Euthymia
Ecstasy
Alexithymia

Pervasive emotional state prevailing over a longer period	Choose...
Inability to verbally describe emotional feelings	Choose...
Inability to experience pleasure	Choose...
Subjective and immediate experience of emotion at a given time	Choose...
Normal range of mood, with absence of depressed or elevated mood	Choose...

Explanation:

Mood is the pervasive and sustained feeling tone that is experienced internally and that, in the extreme, can markedly influence virtually all aspects of a person's behaviour and perception of the world.

Alexithymia is the inability to describe one's subjective emotional experiences verbally. It may be a personality characteristic but is also associated with somatisation.

Anhedonia is the feeling of absent or significantly diminished enjoyment of previously pleasurable activities. It is a core symptom of depressive illness and is also a negative symptom of schizophrenia.

Affect is the subjective and immediate experience of emotion attached to ideas or mental representations of objects. Affect has outward manifestations that may be classified as restricted, blunted, flattened, broad, labile, appropriate, or inappropriate.

Euthymia is normal range of mood, implying absence of depressed or elevated mood.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 1135, 1139, 1142.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 102.

The correct answer is: Pervasive emotional state prevailing over a longer period → Mood, Inability to verbally describe emotional feelings → Alexithymia, Inability to experience pleasure → Anhedonia, Subjective and immediate experience of emotion at a given time → Affect, Normal range of mood, with absence of depressed or elevated mood → Euthymia

Question **12**

Not answered

Marked out of 4.00

Choose...
Alexithymia
Anxiety
Euthymia
Labile Affect
Blunted affect
Agitation
Euphoria
Anhedonia
Incongruent Affect
Elation

Affective disturbances

Identify the terms commonly used in psychopathology using the definitions listed.

Severe reduction in the intensity of affect	Choose...
Rapid and abrupt changes in affect unrelated to external stimuli	Choose...
Objective impression that the displayed affect is not consistent with the current thoughts or actions	Choose...
An unpleasant emotional state associated with psychophysiological changes in response to an intrapsychic conflict	Choose...

Explanation:

Blunting of affect is characterised by loss of the normal degree of emotional sensitivity and sense of appropriate emotional response to events

Lability of affect is defined as rapid and abrupt changes in emotion largely unrelated to external stimuli. These shifts occur without warning. Lability is most common in mixed affective states and in mania. Those with personality disorder of the borderline type may also exhibit lability of affect and describe rapid shifts in mood for little or no reason.

Incongruity of affect refers to the objective impression that the displayed affect is not consistent with the current thoughts or actions (e.g. laughing while discussing traumatic experiences).

Anxiety is an unpleasant affective state and a simple definition is 'fear for no adequate reason'. Sometimes the anxiety is accompanied by physical symptoms such as palpitations, sweating, difficulty breathing, etc.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 103, 113.

Casey P, Kelly B. Fish's clinical pathology: Signs and symptoms in psychiatry, 4th Edition. 2019. pp 73, 76.

The correct answer is: Severe reduction in the intensity of affect → Blunted affect, Rapid and abrupt changes in affect unrelated to external stimuli → Labile Affect, Objective impression that the displayed affect is not consistent with the current thoughts or actions → Incongruent Affect, An unpleasant emotional state associated with psychophysiological changes in response to an intrapsychic conflict → Anxiety

Question **13**

Not answered

Marked out of 5.00

Disturbances of attention

Choose the most appropriate term that illustrates each of the following disturbances of attention

Narrowly focused attention and altered consciousness seen in dissociative states — Choose...

Abnormal drowsiness — Choose...

Drowsiness, confusion and falls at night time in older people often with reduced cognitive reserve — Choose...

Disturbed consciousness with hallucinations — Choose...

Excessive attention to all internal and external stimuli — Choose...

Choose...
Coma
Distractibility
Twilight state
Hypervigilance
Somnolence
Selective inattention
Sundowning
Trance

Explanation:

Trance is a sleep-like state of reduced consciousness and activity.

Somnolence is pathological sleepiness or drowsiness from which one can be aroused to a normal state of consciousness.

Sundowning is characterised by drowsiness, confusion, ataxia, and falls. It occurs in older people who are overly sedated and in patients with dementia who react adversely to even a small dose of a psychoactive drug. The syndrome also occurs in demented patients when external stimuli, such as light and interpersonal orienting cues, are diminished.

Twilight state is characterised by disturbed consciousness with hallucinations.

Hypervigilance is characterised by excessive attention to and focus on all internal and external stimuli. It is usually seen in delusional or paranoid states, and also in PTSD.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 236, 1140, 1144-1145.

The correct answer is: Narrowly focused attention and altered consciousness seen in dissociative states

→ Trance, Abnormal drowsiness → Somnolence, Drowsiness, confusion and falls at night time in older people often with reduced cognitive reserve → Sundowning, Disturbed consciousness with hallucinations → Twilight state, Excessive attention to all internal and external stimuli → Hypervigilance

Question **14**

Not answered

Marked out of 5.00

Phobias

Identify the terms used for different specific phobias

Fear of open spaces	Choose...
Fear of enclosed spaces	Choose...
Fear of heights	Choose...
Fear of snakes	Choose...
Fear of number thirteen	Choose...

Choose...
Thalassophobia
Amathophobia
Acrophobia
Triskaidekaphobia
Odynophobia
Gynophobia
Claustrophobia
Ophidiophobia
Cynophobia
Agoraphobia

Explanation:

Fear of open or crowded spaces is agoraphobia. Fear of enclosed spaces is claustrophobia. Fear of heights is acrophobia. Fear of snakes is ophidiophobia. Fear of number thirteen is triskaidekaphobia. Cynophobia is fear of dogs.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 376.

The correct answer is: Fear of open spaces → Agoraphobia, Fear of enclosed spaces → Claustrophobia, Fear of heights → Acrophobia, Fear of snakes → Ophidiophobia, Fear of number thirteen → Triskaidekaphobia

Question **15**

Not answered

Marked out of 4.00

Pathology of familiarity

Identify the terms used to describe each of the following

Feeling of unfamiliarity with a previously experienced situation	Choose...
Illusion of auditory recognition	Choose...
Regarding a new situation as a repetition of a previous experience	Choose...
Regarding a new thought as a repetition of a previous thought	Choose...

Choose...
Derealization
Jamais vu
Déjà pensee
Confabulation
Déjà entendu
Depersonalization
Déjà vecu
Derailment
Anterograde amnesia
Déjà vu

Explanation:

Jamais vu is the sensation that events or situations are unfamiliar, although they have been experienced before. Déjà vu is a sense that events being experienced for the first time have been experienced before. Both déjà vu and jamais vu can be everyday experiences, but also non-specific symptoms of a number of disorders, including temporal lobe epilepsy, schizophrenia, and anxiety disorders. Deja entendu is the illusion that what one is hearing one has heard previously. Deja pense is a condition in

which a thought never entertained before is incorrectly regarded as a repetition of a pervious thought.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 105, 113.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 1137.

The correct answer is: Feeling of unfamiliarity with a previously experienced situation → Jamais vu, Illusion of auditory recognition → Déjà entendu, Regarding a new situation as a repetition of a previous experience → Déjà vu, Regarding a new thought as a repetition of a previous thought → Déjà pensee

Question **16**

Not answered

Marked out of 4.00

Content of delusions

Identify the type of delusions for each of the examples given below

A 46-year-old depressed woman says 'I am dead.' Choose...

A 76-year-old woman with dementia, on meeting her husband, says 'You are not my husband. You are an impostor who looks like him.' Choose...

A 66-year-old man with psychosis mentions, 'I am constantly followed by people I know, but I can't recognise them as they are always in disguise.' Choose...

A 47-year-old depressed alcoholic is agitated and is convinced that his wife is having an affair with his neighbour. Choose...

Choose...
Ekbom syndrome
Mirrored self-identification
Cotard delusion
Othello syndrome
Autochthonous delusions
Capgras delusion
Folie a deux
Fregoli delusion
Paranoid delusions
De Clerambault delusion

Cotard syndrome is a presentation of psychotic depressive illness seen particularly in elderly people. There is a combination of severely depressed mood with nihilistic delusions and/or hypochondriacal delusions. The patient may state that they are already dead and should be buried, that their insides have stopped working and are rotting away, or that they have stopped existing altogether.

Capgras syndrome is a type of delusional misidentification in which the patient believes that a person known to them has been replaced by a ‘double’ who is to all external appearances identical but is not the ‘real person’. This syndrome can occur both in psychiatric disorders, mainly psychotic disorders (especially schizophrenia) as well as major affective disorders, and neurocognitive disorders (e.g., post-traumatic encephalopathy, epilepsy, cerebrovascular disease, dementia, and delirium/acute confusional state).

Fregoli syndrome is a type of delusional misidentification in which the patient believes that strangers have been replaced with familiar people.

Morbid jealousy (‘delusional jealousy’) is a delusional belief that one’s partner is being unfaithful. This can occur as part of a wider psychotic illness, secondary to organic brain damage, associated with alcohol dependence, or as a monosymptomatic delusional disorder (‘Othello syndrome’).

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 104-106, 111.

Karakasi MV et al. In fear of the most loved ones. A comprehensive review on Capgras misidentification phenomenon and case report involving attempted murder under Capgras syndrome in a relapse of a schizophrenia spectrum disorder. Journal of forensic and legal medicine. 2019 Aug 1;66:8-24.

The correct answer is: A 46-year-old depressed woman says 'I am dead.' → Cotard delusion, A 76-year-old woman with dementia, on meeting her husband, says 'You are not my husband. You are an impostor who looks like him.' → Capgras delusion, A 66-year-old man with psychosis mentions, 'I am

constantly followed by people I know, but I can't recognise them as they are always in disguise.' → Fregoli delusion, A 47-year-old depressed alcoholic is agitated and is convinced that his wife is having an affair with his neighbour. → Othello syndrome

Question **17**

Not answered

Marked out of 3.00

Examples of formal thought disorders

Using the descriptions given below, identify the correct psychiatric terms used to describe them from the list shown.

A 43-year-old lady with bipolar disorder says 'I ate my food, mood, rude, nude.' Choose...

A 33-year-old schizophrenic says 'All is nothing and under nothing twists.' Choose...

A 19-year-old gentleman with paranoid psychosis says ' I have a helopantic under my foot.' Choose...

Choose...
Neologism
Incoherence (word salad)
Clanging
Derailment
Perseveration
Flight of ideas
Circumstantiality
Echolalia
Tangentiality

Explanation:

Clang association is an abnormality of speech where the connection between words is their sound, rather than their meaning. This may occur during manic flight of ideas.

Word salad is the most severe degree of schizophrenic thought disorder, in which no connection of any kind of understandable between sequential words and phrases the patient uses. It is also called schizophasia.

Neologisms may be new words that are constructed by the patient or ordinary words that are used in a new way. The term ‘neologism’ is usually applied to new word formations produced by individuals with schizophrenia. When an individual with schizophrenia produces a new word, it may be completely new and its derivation cannot be understood; it may be a distortion of another word; or it may be a word that has been incorrectly constructed by the faulty use of the accepted rules of word formation.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 104, 119.

Casey P, Kelly B. Fish’s clinical pathology: Signs and symptoms in psychiatry, 4th Edition. 2019. p. 57.

The correct answer is: A 43-year-old lady with bipolar disorder says 'I ate my food, mood, rude, nude.' → Clanging, A 33-year-old schizophrenic says 'All is nothing and under nothing twists.' → Incoherence (word salad), A 19-year-old gentleman with paranoid psychosis says ' I have a helopantic under my foot.' → Neologism

Question **18**

Not answered

Marked out of 5.00

Motor behaviour

Choose the name of the motor phenomenon from the explanation given below:

Pathological imitation of movements of one person by another Choose...

Increased resting muscle tone, not present during active or passive movements Choose...

Ingrained habitual voluntary movement which appear to be goal-directed Choose...

Choose...
Catatonia
Stereotypy
Automatic obedience
Echopraxia
Waxy flexibility
Echolalia
Akathisia
Mannerism
Negativism
Mutism

Subjective feeling of muscular tension secondary to antipsychotic use	Choose...
Repetitive, fixed pattern of physical action which is not goal directed	Choose...

Explanation:

Echopraxia is the repeating or mimicking of the movements of another person.

Catatonia is characterised by increased resting muscle tone that is not present on active or passive movement (in contrast to the rigidity associated with Parkinson's disease and extra-pyramidal side effects).

A mannerism is the abnormal and occasionally bizarre performance of a voluntary, goal-directed activity (e.g. a conspicuously dramatic manner of walking).

Akathisia is a subjective feeling of motor restlessness manifested by a compelling need to be in constant movement. It may be seen as an adverse effect of antipsychotic medication. It may be mistaken for psychotic agitation.

Stereotypy is a repetitive and bizarre movement which is not goal-directed (in contrast to mannerism, which is goal-directed). A stereotypy may have delusional significance to the patient and is sometimes seen in schizophrenia.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 1135, 1139.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 104, 114, 118.

The correct answer is: Pathological imitation of movements of one person by another → Echopraxia, Increased resting muscle tone, not present during active or passive movements → Catatonia, Ingrained habitual voluntary movement which appear to be goal-directed → Mannerism, Subjective feeling of muscular tension secondary to antipsychotic use → Akathisia, Repetitive, fixed pattern of physical action which is not goal directed → Stereotypy

Question **19**

Not answered

Marked out of 5.00

Perceptual errors in psychiatric disorders

Which of the listed descriptive psychopathological terms refers to the following symptoms?

A 43-year-old lady with psychotic depression mentions 'I hear the voice of my long-dead father, as if he were talking to me now, when I hear water running from the bath tap'.	Choose...
A 48-year-old schizophrenic inpatient says that he can hear his father's voice who is talking '20 miles from the ward'.	Choose...
A 22-year-old woman with schizophrenia describes the sensation that somebody is touching her body in intimate places.	Choose...
A 45-year-old schizophrenic lady says that she sees red flames when she hears a song.	Choose...
A 47-year-old woman with schizophrenia says that she is hearing her own thoughts aloud.	Choose...

Choose...
Charles Bonnet syndrome
Extracampine hallucination
Hygric hallucination
Synaesthesia
Functional hallucination
Gedankenlautwerden
Haptic hallucination
Completion illusion
Pareidolic illusion
Reflex hallucination

Functional hallucinations: An external stimulus provokes hallucination, and both hallucination and stimulus are in same modality but separately perceived. e.g voices heard whenever the noise of water

running through the tap is heard.

Extracampine hallucinations occur outside the normal field of perception e.g. images seen behind your back, under your sternum, etc. They occur in schizophrenia and epilepsy.

Haptic hallucination (Touch) is a type of superficial somatic hallucination.

When hallucinations in one modality are provoked reflexively by a stimulus in another modality. e.g., seeing an angel or flames whenever listening to music, the term reflex hallucination is used. The scenario in the question is unlikely to be synaesthesia as both precepts are experienced (it is not the song/music that is experienced as colour red, but after the song is heard, the flames are perceived - therefore there is no synchronous occurrence).

One special type of hallucination, which is characteristic of schizophrenia, is hearing one's own thoughts being spoken aloud. This is known as 'Gedankenlautwerden' in German and 'Echo de la pense' in French.

The correct answer is: A 43-year-old lady with psychotic depression mentions 'I hear the voice of my long-dead father, as if he were talking to me now, when I hear water running from the bath tap'. → Functional hallucination, A 48-year-old schizophrenic inpatient says that he can hear his father's voice who is talking '20 miles from the ward'. → Extracampine hallucination, A 22-year-old woman with schizophrenia describes the sensation that somebody is touching her body in intimate places. → Haptic hallucination, A 45-year-old schizophrenic lady says that she sees red flames when she hears a song. → Reflex hallucination, A 47-year-old woman with schizophrenia says that she is hearing her own thoughts aloud. → Gedankenlautwerden

Question **20**

Not answered

Marked out of 3.00

Illusions

Which of the listed descriptive psychopathological terms refers to the following symptoms?

A 48-year-old man says that he can see a ghost's picture on his carpet that has plenty of cigarette burns — Choose...

A 47-year-old depressed woman is reading the word 'deed' as 'dead.' — Choose...

A 26-year-old man hurrying to post office reads 'LOCK' as 'where — Choose...

Choose...
Functional hallucination
Pareidolic illusion
Kinaesthetic hallucination
Hypnagogic hallucination
Hypnapompic hallucination
Complete illusion
Affect illusion
Thermic hallucination
Reflex hallucination
Synaesthesia

Explanations: In pareidolic illusions, the objects are formed from ambiguous stimuli, coloured by prevailing emotion; not entirely due to inattention or affective change. On paying extra effort, the object intensifies and does not disappear.In affect illusion, the prevailing emotional state leads to misperceptions. It disappears on focussing the object with additional concentration. In completion illusions, the stimulus that does not form a complete object might be perceived to be complete, due to inattention. As a rule, this illusion disappears on concentration.

The correct answer is: A 48-year-old man says that he can see a ghost's picture on his carpet that has plenty of cigarette burns → Pareidolic illusion, A 47-year-old depressed woman is reading the word 'deed' as 'dead.' → Affect illusion, A 26-year-old man hurrying to post office reads 'LOCK' as 'where → Complete illusion

Question **21**

Not answered

Marked out of 4.00

Disorders of volition/action

Which of the listed descriptive psychopathological terms refers to the following symptoms?

A patient says 'someone is using telepathy to make my hands

Choose...
Somatic hallucination
Made affect
Delusional perception
Compulsion
Referential delusion
Made volition
Delusional misrepresentation
Made impulse
Somatic passivity

tremble' though no obvious tremors are noted. Choose...

'I know it's wrong to do it, and in fact I do not want to do it. However, despite my resistance, they force me to do it.' Choose...

'When I heard the station master's whistle, I came to know that there is a plot to finish me off.' Choose...

A patient says 'I feel unpleasant due to insects crawling under my skin.' Choose...

Case 1 describes somatic passivity, which is an experience of sensations (trembling) on the body caused by an external agency (telepathy). It is one of the first rank symptoms of schizophrenia.

Case 2 describes made impulse (someone controlling the desire to act- not completed act but the drive. If the action has been carried out, the patient admits to ownership of act, not the impulse behind it).

Case 3 is an example of delusional perception, which is to attribute a new meaning, usually in the sense of self-reference, to a typically perceived object, provided that the new meaning cannot be explained by the person's mood or disturbed perception.

Case 4 describes a type of superficial somatic hallucinations called as formication. Formication (formic acid - from ants) is a special type of haptic hallucination where an unpleasant sensation of little animals or insects crawling under one's skin is perceived. It is seen in delirium tremens and cocaine intoxication.

The correct answer is: A patient says 'someone is using telepathy to make my hands tremble' though no obvious tremors are noted. → Somatic passivity, 'I know it's wrong to do it, and in fact I do not want to do it. However, despite my resistance, they force me to do it.' → Made impulse, 'When I heard the station master's whistle, I came to know that there is a plot to finish me off.' → Delusional perception, A patient says 'I feel unpleasant due to insects crawling under my skin.' → Somatic hallucination

Question **22**

Not answered

Marked out of 9.00

HiY Assessment CXEMI072

Thought disorders

Identify the specific disorders of thought for each group given below using the drop-down list.

Loosened association
Thought alienation
Delusions of persecution
Crowding of thought
Suicidal thoughts
Formication
Obsessive thought
Poverty of speech
Delusional perception
Tangentiality

1. Disorders of stream of thought (Choose two answers)

2. Disorders of the possession of thoughts (Choose two answers)

3. Disorders of the content of thought (Choose three answers)

4. Disorders of form of thought (choose two answers)

Disorders of the stream of thought includes crowding, poverty and pressured thoughts.

Disorders of the possession of thoughts include obsessive thoughts and thought alienation.

Disorders of the content of thoughts include primary delusions (delusional mood, delusional perception, sudden delusional ideas), secondary delusions, suicidal thoughts, etc.

Disorders of the form of thought include loosened associations, tangentiality, etc.

The correct answer is:
Disorders of stream of thought (Choose two answers) - Crowding of thought, Poverty of speech,

Disorders of the possession of thoughts (Choose two answers) - Thought alienation,Obsessive thoughts

Disorders of the content of thought (Choose three answers) - Delusional perception, Delusion of persecution, Suicidal thoughts

Disorders of form of thought (choose two answers)- Loosened association, Tangentiality

Question **23**

Not answered

Marked out of 5.00

Disorders of speech

Identify the correct term that represents each of the following speech disorders

Inability to vocalize. Choose...

Disorder of articulation. Choose...

The normal flow of speech is interrupted by pauses or by the repetition of fragments of the word. Choose...

Repetition of last uttered word, without any apparent purpose. Choose...

Repetition of last syllable of a word. Choose...

Choose...
Dysarthria
Logorrhoea
Stammering
Elective mutism
Alogia
Palilalia
Logoclonia
Aphonia
Akinetic mutism

Explanation:

Aphonia is loss of the ability to vocalise. It may occur with structural disease affecting the vocal cords directly, the ninth cranial nerve, or higher centres. It may also occur in functional illness where the underlying vocal cord function is normal. This can be demonstrated by asking the patient to cough – a normal cough demonstrates the ability of the vocal cords to oppose normally.

Dysarthria refers to difficulty in articulation, the motor activity of shaping phonated sounds into speech, not in word finding or in grammar.

In stammering the normal slow of speech is interrupted by pauses or by the repetition of fragments of the word. Grimacing and tic-like movements of the body are often associated with stammer.

Logoclonia refers to meaningless repetition of a syllable, especially an end syllable of a word. It was first reported among individuals with dementia by Kraepelin in 1910 who described is as a severe linguistic dysfunction observed in the advanced stages of early-onset Alzheimer disease. Logoclonia has been described by several neurologists since then and has often been linked to Alzheimer disease. Palilalia is similar to logoclonia and has been associated with Parkinson disease and progressive supranuclear palsy. Palilalia, however, refers to the repetition of an entire word, but not a syllable.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 103.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 1138.

Casey P, Kelly B. Fish's clinical pathology: Signs and symptoms in psychiatry, 4th Edition. 2019. p. 56.

Nakagawa Y et al. Logoclonia might be a Characteristic of Logopenic Variant Primary Progressive Aphasia at an Advanced Stage: Potential Mechanisms Underlying Logoclonia. Journal of Alzheimer's Disease. 2019 Jan 1;70(2):515-24.

The correct answer is: Inability to vocalize. → Aphonia, Disorder of articulation. → Dysarthria, The normal flow of speech is interrupted by pauses or by the repetition of fragments of the word. → Stammering, Repetition of last uttered word, without any apparent purpose. → Palilalia, Repetition of last syllable of a word. → Logoclonia

Question **24**

Not answered

Marked out of 4.00

Aphasia

Identify the correct terms that represent each of the following speech disorder

The ability to speak is grossly impaired, but comprehension is intact. Choose...

Speech is fluid and spontaneous, but there is loss of ability to comprehend the meaning of words. Choose...

Patient can speak, read & write fluently, but comprehension is impaired only for spoken language. Choose...

The patient can speak well and comprehend what is spoken, and can also write spontaneously and to dictation, but reading comprehension is impaired. Choose...

Choose...
Broca aphasia
Nominal aphasia
Coprophasia
Pure word blindness
Amnestic aphasia
Wernicke aphasia
Pure word dumbness
Syntactical aphasia
Pure word deafness

Explanation:

Broca aphasia (also called non-fluent or expressive aphasia) had traditionally been characterised by non-fluent speech and impaired repetition but intact auditory comprehension. It has long been thought to be associated with damage to Broca area (i.e. left inferior frontal convolution) or Brodmann area 44.

Wernicke aphasia (also called fluent or receptive aphasia) is characterised by fluent speech, impaired comprehension, and somewhat impaired repetition. It has been associated with damage to Wernicke area in the region of the superior temporal gyrus. The impaired ability to comprehend language directly affects the individual's ability to self-monitor language output and may be related to a breakdown of the syntactic structure of language.

Pure word deafness (also called subcortical auditory dysphasia) denotes the inability to understand speech in the face of spared reading comprehension, writing, and speech production. Hearing remains intact for voices and other sounds, but there is an inability to recognise speech.

Pure word blindness (also known as 'pure alexia', 'alexia without agraphia', and 'agnostic alexia') is characterised by various degrees of impaired word reading with no deficits in the production or comprehension of oral language, and a preserved ability to write spontaneously or on dictation.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 48.

Maffei C et al. Pure word deafness following left temporal damage: Behavioral and neuroanatomical evidence from a new case. Cortex. 2017 Dec 1;97:240-54.

Martinaud O. Visual agnosia and focal brain injury. Revue neurologique. 2017 Jul 1;173(7-8):451-60.

The correct answer is: The ability to speak is grossly impaired, but comprehension is intact. → Broca aphasia, Speech is fluid and spontaneous, but there is loss of ability to comprehend the meaning of words. → Wernicke aphasia, Patient can speak, read & write fluently, but comprehension is impaired only for spoken language. → Pure word deafness, The patient can speak well and comprehend what is spoken, and can also write spontaneously and to dictation, but reading comprehension is impaired. →

Pure word blindness

Question **25**

Not answered

Marked out of 5.00

Pioneers in psychopathology

Match each concept below with the pioneer from the list shown:

Founder of psychopathology	Choose...
Concrete thinking	Choose...
First rank symptoms	Choose...
Loosening of association	Choose...
Overinclusive thinking	Choose...

Choose...
Fish
Jung
Sims
Cameron
Bleuler
Carl Schneider
Goldstein
Jaspers
Freud
Kurt Schneider

Karl Jasper is widely considered a major figure in philosophy and psychiatry. He is the founder of psychopathology. It is important to know about Jaspers and his method of enquiry into psychiatry for one to understand psychopathology. He introduced phenomenology, which was a long regarded as a method of philosophic enquiry, to psychiatry.

Concrete thinking is associated with Goldstein. He claimed that both in schizophrenia and in organic brain diseases, a loss of the abstract attitude takes place so that the thinking becomes concrete i.e. the patient is unable to free himself from the superficial concrete aspects of thinking.

Kurt Schneider, a German psychiatrist and a pupil of Karl Jaspers, pointed out certain symptoms as being characteristic of schizophrenia and therefore exhibiting a "first-rank" status in the hierarchy of potentially diagnostic symptoms. The "first-rank" symptoms (FRS) have played an extremely important role in the recent diagnostic systems: in the International Statistical Classification of Diseases, Tenth Revision (ICD-10) as well as in Diagnostic and Statistical Manual of Mental Disorder, (DSM- for versions 3 and 4), the presence of one FRS is symptomatically sufficient for the schizophrenia diagnosis (not for DSM-5) but FRS are not essential to diagnose schizophrenia.

Bleuler proposed four fundamental symptoms of schizophrenia which includes ambivalence, autism, affect disturbances and loosening of associations.

Cameron proposed four characteristic formal thought disorders which include over-inclusive thinking.

The correct answer is: Founder of psychopathology → Jaspers, Concrete thinking → Goldstein, First rank symptoms → Kurt Schneider, Loosening of association → Bleuler, Overinclusive thinking → Cameron

Question **26**

Not answered

Marked out of 9.00

Dementia

Choose salient features for each item below from the list shown:

A salient feature of Lewy body dementia	Choose...
A salient feature of frontotemporal dementia	Choose...
A salient feature of vascular dementia	Choose...
A salient feature of CJD	Choose...

Choose...
Personality change
Visual hallucinations
Striking insight loss
Seizures
Weight loss
History of TIAs
Stepwise cognitive deterioration
Auditory hallucinations
Emotional blunting
Myoclonus

Explanation:

Dementia with Lewy bodies is characterised by fluctuating cognitive performance and consciousness and early sparing of memory; Parkinsonism; complex hallucinations (visual 60% and auditory 20%); depressive symptoms; recurrent falls/syncope; transient disturbances of consciousness; antipsychotic sensitivity.

ICD-11 characterises frontotemporal dementia (FTD) as a group of primary neurodegenerative disorder primarily affecting the frontal and temporal lobes. Onset is typically insidious with a gradual and worsening course. Several syndromic variants are described that include presentations with predominantly marked personality and behavioural changes or with predominantly language deficits, or with a combination of these deficits. Memory function, psychomotor speed, as well as visuoperceptual and visuospatial abilities often remain relatively intact, particularly during the early stages of the disorder.

Features that suggest a vascular cause of cognitive impairment include: sudden onset, stepwise deterioration, and risk factors for cardiovascular disease. In multi-infarct dementia, multiple strokes lead to stepwise deterioration in cognitive function. Between strokes, there are periods of relative stability.

The clinical picture of Creutzfeldt-Jakob disease is one of rapidly progressive dementia, cerebellar and extra-pyramidal signs, myoclonus, and death within a year.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 162-164, 168.

International Statistical Classification of Diseases and Related Health Problems (**11th** ed,; **ICD-11**; World Health Organization, 2020)

The correct answer is: A salient feature of Lewy body dementia → Visual hallucinations, A salient feature of frontotemporal dementia → Personality change, A salient feature of vascular dementia → Stepwise cognitive deterioration, A salient feature of CJD → Myoclonus

Question **27**

Not answered

Marked out of 4.00

Psychosomatic disorders

Identify the diagnosis for each of the following clinical situations:

A 30-year-old woman presents with hyperpigmented buccal mucosa. She has also lost weight of late, which she considers on a positive note. She reports some nausea in the mornings, abdominal pains, feeling dizzy on and off and feeling confused at times. — Choose...

A 55-year-old woman presents with weight loss, foul-smelling bulky stools, abdominal pain, bloating, mouth ulcers and vomiting. Pathology shows villous atrophy on jejunal biopsy. — Choose...

A 24-year-old lady has weight loss and irregular periods. She also has tremors and frequent stools of normal consistency. She is not comfortable with the heating even in the coldest of days. — Choose...

A 27-year-old woman has weight gain and hirsutism. She is also clinically depressed and has a history of hypertension. — Choose...

Choose...
Binge eating disorder
Autoimmune lupus
Hypothyroidism
Celiac disease
Cushing's disease
Hyperthyroidism
Addison's disease
Irritable bowel syndrome
Lymphoma (Hodgkin's)

Addison's disease: Clinical features include weakness, abdominal pain, anorexia, weight loss, constipation, myalgia and depression. Signs include hyperpigmentation (palmar creases, buccal mucosa), vitiligo and postural hypotension.

Coeliac disease: Clinical presentations include abdominal pain, steatorrhoea, bloating, nausea/vomiting, aphthous ulcers, angular stomatitis, weight loss and fatigue. Jejunal biopsy shows villous atrophy, reversing on gluten free diet.

Hyperthyroidism: Clinical symptoms include weight loss despite increased appetite, heat intolerance, sweating, diarrhoea, tremor, irritability, emotional lability, psychosis, oligomenorrhoea, and itch. Signs include atrial fibrillation, warm peripheries, fine tremor, goitre, palmar erythema, hair thinning and lid lag.

Cushing's syndrome: Clinical features include weight gain, menstrual irregularity, amenorrhea, and hirsutism in females, impotence in males, depression, muscle weakness and fractures. Signs include tissue wasting, myopathy, thin skin, purple abdominal striae, osteoporosis, hirsutism, hypertension, hyperglycemia, predisposition to infection and bad wound healing.

The correct answer is: A 30-year-old woman presents with hyperpigmented buccal mucosa. She has also lost weight of late, which she considers on a positive note. She reports some nausea in the mornings, abdominal pains, feeling dizzy on and off and feeling confused at times. → Addison's disease, A 55-year-old woman presents with weight loss, foul-smelling bulky stools, abdominal pain, bloating, mouth ulcers and vomiting. Pathology shows villous atrophy on jejunal biopsy. → Celiac disease, A 24-year-old lady has weight loss and irregular periods. She also has tremors and frequent stools of normal consistency. She is not comfortable with the heating even in the coldest of days. → Hyperthyroidism, A 27-year-old woman has weight gain and hirsutism. She is also clinically depressed and has a history of hypertension. → Cushing's disease

Question **28**

Not answered

Marked out of 4.00

Terms related to memory

For each of the description below chose one most appropriate term from the above list

Reintegrating events using a variety of components — Choose...

Modifying memory in terms of attitudes and emotions — Choose...

Absence of the normal feeling of familiarity that accompanies memory when visiting a place for the second time — Choose...

Feeling of familiarity that accompanies return of stored material to consciousness — Choose...

Choose...
Recapitulation
Retrospective falsification
Confabulation
Delusional elaboration
Recollection
Jamais vu
Recognition
Registration
Déjà vu

Explanation:

Recollection entails the extension of memory back to events in the past. The actual reconstruction of these events comes through reminiscence, associations, and autobiographical linking of events.

Retrospective falsification is the unconscious alteration or distortion of memories to make them satisfy present wishes or needs.

Jamais vu is the sensation that events or situations are unfamiliar, although they have been experienced before. It can be an everyday experience, but also a non-specific symptom of a number of disorders, including temporal lobe epilepsy, schizophrenia, and anxiety disorders.

Recognition is the act or process of perceiving or identifying information as matching or being the same as information that has been remembered.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 745.

Colman AM. Oxford Dictionary of Psychology, 4 th Edition. 2015. pp. 640, 657.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 113.

The correct answer is: Reintegrating events using a variety of components → Recollection, Modifying

memory in terms of attitudes and emotions → Retrospective falsification, Absence of the normal feeling of familiarity that accompanies memory when visiting a place for the second time → Jamais vu, Feeling of familiarity that accompanies return of stored material to consciousness → Recognition

Question **29**

Not answered

Marked out of 5.00

Psychopathology of memory

Choose the appropriate terms associated with each of the following descriptions:

Falsification of memory occurring in clear consciousness seen in alcoholic subjects	Choose...
Fluent possible lying seen in dissocial personality disorders	Choose...
Wandering away from surroundings and subsequent amnesia for the episode	Choose...
Hysterical dissociative disorder presenting with approximate answers seen in prisoners	Choose...
Occurs as part of a dissociation reaction with narrow consciousness and loss of personal identity	Choose...

Choose...
Lethologica
Ganser's syndrome
Repression
Fugue
Dissociative trance
Eidetic images
Pseudologiafantastica
Confabulation
Possession states
Paramnesia

Confabulation is a falsification (or temporal confusion) of memory occurring in clear consciousness associated with organic states. Suggestibility is a prominent feature of confabulation. It is often described in Korsakoff syndrome.

In pseudologia fantastica, there is a fluent but plausible (as opposed to fantastic) lying. It is usually associated with dissocial or histrionic personality disorders, and also reported in Munchausen syndrome.

In a dissociative fugue, wandering away from surroundings and subsequent amnesia for the episode can be seen. There are marked memory loss and loss of identity, but the patient can carry out complicated patterns of behaviour and maintains self-care.

Ganser's syndrome, considered as a hysterical dissociative disorder, includes approximate answers, clouding of consciousness with disorientation, psychogenic, physical symptoms - analgesia & hyperaesthesia, and pseudohallucinations the latter is not always present). Possession states can occur as a part of dissociation or in normal religious experiences, or under hypnosis. Consciousness is altered in dissociative states. There is the loss of personal identity and replaced with another identity.

The correct answer is: Falsification of memory occurring in clear consciousness seen in alcoholic subjects → Confabulation, Fluent possible lying seen in dissocial personality disorders → Pseudologiafantastica, Wandering away from surroundings and subsequent amnesia for the episode → Fugue, Hysterical dissociative disorder presenting with approximate answers seen in prisoners → Ganser's syndrome, Occurs as part of a dissociation reaction with narrow consciousness and loss of personal identity → Possession states

Question **30**

Not answered

Marked out of 6.00

Flag question

HiY Assessment CXEMI081

Motor Symptoms

From the above list chose the most appropriate terms describing each of the following clinical observations

1. A patient moves his arm on slightest pressure regardless of any instruction given

Ambitendency
Gegenhalten
Mitgehen
Cataplexy
Catalepsy
Waxy flexibility
Automatic obedience,
Stereotypy
Mannerisms

2. Despite being asked to comply with, a patient resists with same pressure an attempt to move his arm.

3. When asked to put out his hand, a patient extends, withdraws and then extends his hand repeatedly for 23 times before allowing it to rest on the examiner's hand.

4. On hearing a sudden noise, a patient feels that his knees are buckling. He falls and hurts himself.

5. A patient allows his arms to be manipulated freely without much resistance and maintains the posture in which the examiner leaves him for few minutes. (TWO)

Explanation:

Mitgehen or "Angle poise lamp" sign: The patient yields to slightest of pressures, without much resistance, similar to an angle poise lamp that bends easily. This happens even if the patient is instructed to resist any manipulation.

Gegenhalten is a milder form of resistance/negativism. Patients with negativism resist or oppose all passive movements attempted by the examiner. A mild form of such resistance is called Gegenhalten or opposition.

In ambitendency, the patient makes a series of tentative, opposing alternate movements that do not reach the intended goal. This becomes evident when the patient is asked to carry out a motor act. Ambivalence is the inability to make a decision - dilemma of the volitional faculty.

Cataplexy refers to an abrupt loss of muscle tone leading to collapse. It occurs following emotional stress.

In catalepsy, the patient's limbs can be passively moved to any posture which will then be held for a prolonged period of time. It is also called as waxy flexibility (flexibilitas cerea) Here the patient shows wax like plastic 'mouldable' quality. His limbs can be moved by the examiner to occupy certain postures, which are then maintained, even if these are uncomfortable and bizarre

The correct answer is:

A patient moves his arm on slightest pressure regardless of any instruction given - Mitgehen,

Despite being asked to comply with, a patient resists with same pressure an attempt to move his arm.- Gegenhalten,

When asked to put out his hand, a patient extends, withdraws and then extends his hand repeatedly for 23 times before allowing it to rest on the examiner's hand.- Ambitendency,

On hearing a sudden noise, a patient feels that his knees are buckling. He falls and hurts himself.- Cataplexy,

A patient allows his arms to be manipulated freely without much resistance and maintains the posture in which the examiner leaves him for few minutes. (TWO) - Catalepsy, Waxy flexibility

Question **31**

Not answered

Marked out of 5.00

Disorders of perception

Choose the most appropriate term(s) to describe the following bodily sensations:

A 53-year-old schizophrenic says 'My feet on fire.' Choose...

A 47-year-old woman claims 'A dead hand touched me.' Choose...

A 23-year-old man with psychosis was assessed due to decline in his mental state. During assessment, he says 'all my blood has dropped into my legs, and I can feel a water level in my chest.' Choose...

A 27-year-old man with paranoid schizophrenia says 'I thought my life was outside my feet and made them vibrate.' Choose...

A 23-year-old drug addict tells 'I can feel insects crawling under my skin.' Choose...

Choose...
Kinaesthetic hallucination
Hypnagogic hallucination
Haptic hallucination
Affect illusion
Pareidolic illusion
Reflex hallucination
Hypnapompic hallucination
Thermic hallucination
Completion illusion
Hygric hallucination

Explanation:

Tactile hallucinations are hallucinations of touch. Sims classifies tactile hallucinations into three main types: superficial, kinaesthetic and visceral. Sims further divides superficial hallucinations, which affect the skin, into four types: **thermic** (e.g., a cold wind blowing across the face), **haptic** (e.g., feeling a hand brushing against the skin), **hygric** (e.g., feeling fluid such as water running from the head into the stomach), and paraesthetic (pins and needles). **Kinaesthetic** hallucinations affect the muscles and joints, and the patient feels that their limbs are being twisted, pulled, or moved. Vestibular sensations such as sinking in the bed or flying through the air can also be hallucinated and are best regarded as a variant of kinaesthetic hallucinations. **Formication** is a form of tactile (or **haptic**) hallucination in which there is the sensation of numerous insects crawling over the surface of the body. This is not uncommon in acute organic states, including alcohol or drug withdrawal, particularly from cocaine.

Ref: Casey P, Kelly B. Fish's clinical pathology: Signs and symptoms in psychiatry, 4th Edition. 201 pp. 33-34.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 1140.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 111.

The correct answer is: A 53-year-old schizophrenic says 'My feet on fire.' → Thermic hallucination, A 47-year-old woman claims 'A dead hand touched me.' → Haptic hallucination, A 23-year-old man with psychosis was assessed due to decline in his mental state. During assessment, he says 'all my blood has dropped into my legs, and I can feel a water level in my chest.' → Hygric hallucination, A 27-year-old man with paranoid schizophrenia says 'I thought my life was outside my feet and made them vibrate.' → Kinaesthetic hallucination, A 23-year-old drug addict tells 'I can feel insects crawling under my skin.' → Haptic hallucination

Question **32**

Not answered

Marked out of 4.00

Flag question

Thought & speech problems

Identify the correct psychopathological term for each of the following descriptions:

A 27-year-old prisoner with schizophrenia gives approximate answers to simple questions Choose...

A 47-year-old man sustained a head injury make inappropriate comments and cracks jokes Choose...

A 69-year-old man with Parkinson's disease gets stuck on a particular

Choose...
Paragrammatism
Overinclusive thinking
Logorrhoea
Vorbeireden
Verschmelzung
Neologism
Witzelsucht
Concrete thinking
Logoclonia

word of a sentence and repeats it — Choose...

A 23-year-old manic woman has excessive talking that was incoherent and has no logic — Choose...

Vorbeireden is talking past the point, described as formal thought disorder, different from the flight of ideas, which is a disorder of thought stream. It is not exclusively associated with Ganser's syndrome. It is also seen in acute schizophrenia and hebephrenic schizophrenia. Vorbeireden ('talking past the point') is often used interchangeably with vorbeigehen ('going past the point'), although the latter was originally defined as part of the 'Ganser syndrome', whereby some criminals would give incorrect answers ('approximate answers') to simple questions that none the less suggested that the correct answer was known (e.g. saying dogs have five legs.

Witzelsucht (from the German witzeln, meaning to joke or wisecrack, and sucht, meaning addiction) is characterized by a tendency to make puns, tell inappropriate jokes or pointless stories in socially inappropriate situations. Ironically, however, the person is insensitive to humor produced by themselves or others around them. This disorder is most commonly seen in patients with frontal lobe damage, particularly right frontal lobe tumors or trauma.

Logoclonia is a symptom of Parkinson's disease and Alzheimer's dementia where the patient gets stuck on a particular word of a sentence and repeats it.

Logorrhoea is excess speech or verbal diarrhoea, which is a symptom of mania. It involves excessive talking or mumbling monotonously and often incoherently with words that avoid any logic or reason. (Excerpts from Ordered thoughts on thought disorder. Psychiatric Bulletin. (n.d.). Retrieved from http://pb.rcpsych.org/content/29/12/462)

The correct answer is: A 27-year-old prisoner with schizophrenia gives approximate answers to simple questions → Vorbeireden, A 47-year-old man sustained a head injury make inappropriate comments and cracks jokes → Witzelsucht, A 69-year-old man with Parkinson's disease gets stuck on a particular word of a sentence and repeats it → Logoclonia, A 23-year-old manic woman has excessive talking that was incoherent and has no logic → Logorrhoea

Question **33**

Not answered

Marked out of 11.00

Choose...
Humour
Denial
Obsession
Intellectualisation
Regression
Deindividuation

Defence mechanisms - categories

Identify a defence mechanism for each of the following categories

Mature defense — Choose...

Neurotic defense — Choose...

Narcissistic defense — Choose...

Kleinian defense — Choose...

Immature defense — Choose...

Explanation:

Mature defences include altruism, anticipation, asceticism, humour, sublimation, and suppression.

Neurotic defences include controlling, displacement, dissociation, externalisation, inhibition, intellectualisation, isolation, rationalisation, reaction formation, repression, and sexualisation.

Narcissistic defences include projection, denial, and distortion.

Kleinian defences include splitting, introjection, projective identification, denial, omnipotence, and grandiosity (mnemonic: SIPDOG).

Immature defences include acting out, blocking, hypochondriasis, introjection, passive-aggressive behaviour, regression, schizoid fantasy, and somatisation.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 1066-1067.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 888.

The correct answer is: Mature defense → Humour, Neurotic defense → Intellectualisation, Narcissistic defense → Denial, Kleinian defense → Denial, Immature defense → Regression

Question **34**

Not answered

Marked out of 3.00

Problem Workers

There are three people in an office with problems at work, and all of these are the result of long-standing behaviour patterns. Identify the type of personality disorders for each of the following situations

Choose...
Paranoid personality disorder
Histrionic personality disorder
Anxious avoidant personality disorder
Schizoid personality disorder
Schizotypal personality disorder
Anankastic personality disorder
Narcissistic personality disorder
Dependent personality disorder
Borderline personality disorder
Antisocial personality disorder

The manager is rigid about timetables and expects all their colleagues to be like them — Choose...

A secretary who is nervous and keeps to herself, but wants to have friends and a boyfriend — Choose...

A woman who comes to work for a long weekend is annoyed that her office plants have died because nobody watered them. — Choose...

Anankastic personality disorder- frequently perfectionists, often lack clarity in seeing other perspectives or ways of doing things, Rigid attention to detail may prevent them from completing tasks; may not be able to delegate tasks; workaholics.

Avoidant personality disorder - Fears being judged negatively by other people, Feelings of discomfort in group/social settings; May come across as being socially withdrawn and have low self-esteem.

Paranoid personality disorder: Patients often suspect that others are exploiting, harming, or deceiving them without sufficient basis. This personality disorder is characterized by the presence of a pervasive distrust and suspiciousness that others' motives are malevolent.

The correct answer is: The manager is rigid about timetables and expects all their colleagues to be like them → Anankastic personality disorder, A secretary who is nervous and keeps to herself, but wants to have friends and a boyfriend → Anxious avoidant personality disorder, A woman who comes to work for a long weekend is annoyed that her office plants have died because nobody watered them. → Paranoid personality disorder

Question **35**

Not answered

Marked out of 3.00

Flag question

Neurological symptoms

Find one term each to describe the following presentations

A 65-year-old gentleman was unable to identify familiar objects such as key or coins by touch with eyes closed. He has difficulty in describing their shape and texture. However, he identifies them when eyes are open. — Choose...

Choose...
Visuospatial agnosia
Astereognosis
Agraphaesthesia
Anosognosia
Prosopagnosia
Right - left disorientation
Dressing apraxia
Finger agnosia
Constructional apraxia

A 67-year-old woman is unable to recognize the number scratched on her skin without seeing the act of scratching. Choose...

A man has left side hemiplegia after a stroke. He is unaware of his deficit. Choose...

Explanation:

Astereognosis is the inability to recognise familiar objects by touch. **Agraphaesthesia** is the inability to recognise a number or letter scratched onto the skin by touch. **Anosognosia** is the inability to recognise a physical deficit in oneself (e.g. patient denies paralysed limb).

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 1136.

The correct answer is: A 65-year-old gentleman was unable to identify familiar objects such as key or coins by touch with eyes closed. He has difficulty in describing their shape and texture. However, he identifies them when eyes are open. → Astereognosis, A 67-year-old woman is unable to recognize the number scratched on her skin without seeing the act of scratching. → Agraphaesthesia, A man has left side hemiplegia after a stroke. He is unaware of his deficit. → Anosognosia

Question **36**

Not answered

Marked out of 5.00

Choose...
Predictive
Concurrent
Criterion
Face
Content
Incremental

Validity of rating scales

Find one term each to describe the following descriptions of psychometric validity of scales measuring depression

A scale 'X' adds some more information to the other scales measuring depression Choose...

On inspection, the items of a scale 'Y' appears to assess depression Choose...

A scale 'Z' includes an exhaustive coverage of all of the expected symptoms of depression Choose...

Scores from a new scale for depression has a high degree of correlation with the scores from Hamilton depression scale Choose...

Higher scores on the Hamilton depression scale predict the risk of future self-harm in a cohort of untreated patients. Choose...

Face validity refers to the subjective measure of deciding whether the test measures the construct of interest on its face value. e.g., Hamilton depression scale clearly has a face validity for measuring depression; but not for measuring obsessions.

Content validity refers to whether the contents i.e. each subscale, items or elements of the test are in line with the general objectives or specifications the test was originally designed to measure. It assesses the degree of coverage of all domains in relation to the measured condition.

Criterion validity refers to the performance against an external criterion such as another instrument (concurrent) or future diagnostic possibility (predictive).

Predictive validity refers to the ability of a test to predict future group differences according to current group differences in the score. e.g. high aggression score in childhood could predict high criminal incidents in adult life.

Incremental validity refers to the ability of a measure to predict or explain variance over and above other measures.

The correct answer is: A scale 'X' adds some more information to the other scales measuring

depression → Incremental, On inspection, the items of a scale 'Y' appears to assess depression → Face, A scale 'Z' includes an exhaustive coverage of all of the expected symptoms of depression → Content, Scores from a new scale for depression has a high degree of correlation with the scores from Hamilton depression scale → Criterion, Higher scores on the Hamilton depression scale predict the risk of future self-harm in a cohort of untreated patients. → Predictive

Question **37**

Not answered

Marked out of 5.00

Schizophrenia & subtypes

For each of the description below choose one most appropriate diagnostic term using ICD-10 nomenclature:

Choose...
Post schizophrenic depression
Residual Schizophrenia
Schizophreniform psychosis
Schizoid personality disorder
Paranoid schizophrenia
Catatonic schizophrenia
Schizotypal disorder
Simple schizophrenia
Hebephrenic schizophrenia

Prominent delusions and auditory hallucinations	Choose...
Delusions and hallucinations are present at a lesser intensity that the acute phase	Choose...
Marked social withdrawal, loss of initiative & drive, with poorly formed delusions	Choose...
Poor social functioning, affective blunting and marked thought disorder	Choose...
Marked acute onset psychomotor disturbance	Choose...

Paranoid schizophrenia is a common subtype characterized by prominent delusions (often persecutory in content) and hallucinations (usually auditory). Thought disorders, affective abnormality and negative symptoms may be present but not prominent.

Residual schizophrenia consists of a long term but not necessarily irreversible negative symptoms. Delusions and hallucinations are often of low intensity or reduced for at least one year, often following a more florid acute phase.

Simple schizophrenia is characterized by an early onset (usually in the second decade), very insidious and progressive course, and presence of characteristic negative symptoms like marked social withdrawal, loss of initiative and drive and shallow emotional response. Delusions and hallucinations are usually absent; if present they are short lasting and poorly formed.

Hebephrenic schizophrenia is also known as disorganized schizophrenia. It is characterized by marked thought disorder (e.g. severe loosening of associations), inappropriate and blunted affect (e.g. senseless giggling) and abnormal mannerisms (e.g. prolonged mirror gazing). Social and occupational functioning is markedly impaired, often with poor self-care, hygiene, extreme asocial and disorganized behaviour.

Catatonic schizophrenia is a somewhat less common subtype characterized by psychomotor symptoms ranging from violent excitement through posturing, negativism, waxy flexibility, perseveration to stupor. The onset is usually acute with an episodic course and usually a very good degree of recovery.

The correct answer is: Prominent delusions and auditory hallucinations → Paranoid schizophrenia, Delusions and hallucinations are present at a lesser intensity that the acute phase → Residual Schizophrenia, Marked social withdrawal, loss of initiative & drive, with poorly formed delusions → Simple schizophrenia, Poor social functioning, affective blunting and marked thought disorder → Hebephrenic schizophrenia, Marked acute onset psychomotor disturbance → Catatonic schizophrenia

Question **38**

Not answered

Coping mechanisms

For each of the following examples, please select the most likely descriptions of defence mechanisms

Marked out of 4.00

from the list above. Please select only one option, but an option may be used more than once, if required.

Choose...
Projective identification
Intellectualisation
Acting out
Anticipation
Isolation
Reaction formation
Projection
Dissociation
Passive-aggression
Denial

A young man brings his 19-year-old girlfriend who vacantly stares at the wall and doesn't respond to conversations. She has been in this state since they had a serious argument one day ago. — Choose...

After his father's sudden death, Tom took over his family business though he used to detest it for a long time. — Choose...

A 45-year-old woman who was bullied at work comes late to work consistently instead of raising the issue of bullying with the concerned managers. — Choose...

A 45-year-old man holds long-standing suspiciousness towards the police. He becomes concerned and acts in an unusual manner on seeing two policemen patrolling streets. The police then become suspicious of him and arrest him as a precautionary measure. — Choose...

Explanation:

Dissociation is a temporary but drastic modification of character or sense of personal identity to avoid emotional distress. It includes fugue states and conversion reactions.

Reaction formation involves externally expressing attitudes and behaviours that are the opposite of the unacceptable internal impulses (e.g. being extra polite to a person to avoid expressing anger towards them).

Passive aggression is aggression toward an object expressed indirectly and ineffectively through passivity, masochism, and turning against the self.

Projective identification involves behaving towards another in a manner that causes them to take one's on own internal unacceptable ideas and impulses. It should not be confused with projection. Whereas during projection an individual with a certain emotion might perceive someone else as feeling the same way, in projective identification the individual causes the other to feel that emotion (e.g. an angry child behaves in such a way that his mother becomes angry with him).

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 1066-1067.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 892-893.

The correct answer is: A young man brings his 19-year-old girlfriend who vacantly stares at the wall and doesn't respond to conversations. She has been in this state since they had a serious argument one day ago. → Dissociation, After his father's sudden death, Tom took over his family business though he used to detest it for a long time. → Reaction formation, A 45-year-old woman who was bullied at work comes late to work consistently instead of raising the issue of bullying with the concerned managers. → Passive-aggression, A 45-year-old man holds long-standing suspiciousness towards the police. He becomes concerned and acts in an unusual manner on seeing two policemen patrolling streets. The police then become suspicious of him and arrest him as a precautionary measure. → Projective identification

Question **39**

Not answered

Marked out of 5.00

Psychopathological processes

Identify the name of the psychopathological process involved, for each of the following descriptions

Abrupt interruption of thoughts — Choose...

Choose...
Thought blocking
Derailment
Asyndesis
Thought echo
Capgras syndrome
Thought insertion
Substitution
Cotard syndrome
Moria
Couvade syndrome

Deviation of train of thought without blocking	Choose...
Lack of adequate logical connection between successive thoughts	Choose...
A woman believes her intestines have stopped working and are rotting away	Choose...
A conversion symptom is seen in partners of expectant mothers during their pregnancy	Choose...

Explanation:

Thought blocking is a symptom of schizophrenic thought disorder in which the patient experiences a sudden break in the chain of thought.

Derailment is a symptom of schizophrenic thought disorder in which there is a total break in the chain of association between the meaning of the thoughts. The connection between two sequential ideas is apparent neither to the patient nor to the examiner.

Asyndesis (also known as loosening of associations) is a symptom of formal thought disorder in which there is a lack of meaningful connection between sequential ideas.

Cotard syndrome is a presentation of psychotic depressive illness seen particularly in elderly people. There is a combination of severely depressed mood with nihilistic delusions and/or hypochondriacal delusions. The patient may state that they are already dead and should be buried, that their insides have stopped working and are rotting away, or that they have stopped existing altogether.

Couvade syndrome is a conversion symptom seen in partners of expectant mothers during their pregnancy. Symptoms vary, but mimic pregnancy symptoms. It is NOT delusional in nature – the affected individual does not believe that they are pregnant.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 103, 105, 108, 113, 118.

The correct answer is: Abrupt interruption of thoughts → Thought blocking, Deviation of train of thought without blocking → Derailment, Lack of adequate logical connection between successive thoughts → Asyndesis, A woman believes her intestines have stopped working and are rotting away → Cotard syndrome, A conversion symptom is seen in partners of expectant mothers during their pregnancy → Couvade syndrome

Question **40**

Not answered

Marked out of 4.00

Features of personality disorders

Identify the type of personality disorder for each of the following traits

Choose...
Dissocial personality disorder
Paranoid personality disorder
Schizoid personality disorder
Dependent personality disorder
Anankastic personality disorder
Schizotypal personality disorder
Histrionic personality disorder
Borderline personality disorder
Narcissistic personality disorder
Obsessive personality disorder

Excessively emotional with temperamental instability	Choose...
Increased self-referential tendency	Choose...
Excessive sense of entitlement	Choose...
Chronic feelings of emptiness	Choose...

Borderline personality disorder is characterized by emotional instability and lack of impulse control. Chronic feelings of emptiness or boredom with an inability to stay alone are seen in borderline personality disorder.

In paranoid personality disorder, a tendency to hold an excessively self-referential stance is noted.

Narcissistic personality disorder is marked by an excessive sense of entitlement, e.g., unreasonable expectations of especially favorable treatment or automatic compliance of others to his/her wishes.

The correct answer is: Excessively emotional with temperamental instability → Borderline personality disorder, Increased self-referential tendency → Paranoid personality disorder, Excessive sense of entitlement → Narcissistic personality disorder, Chronic feelings of emptiness → Borderline personality disorder

Question **41**

Not answered

Marked out of 3.00

Diagnosing substance use disorders

For each of the following clinical presentations, which one of the listed is the most likely diagnosis?

A 25-year-old woman who is on a controlled drug prescription went on a short holiday for three days and forgot to take her prescribed medication. When she returned home, she developed diarrhoea, running nose and goose flesh. On examination, she had dilated pupils and tachycardia. — Choose...

A 30-year-old gentleman presented to the A&E with paranoid delusions and prominent visual hallucinations of very rapid onset following ingesting of a street drug. On examination, he had tachycardia, hypertension and pupillary dilatation. — Choose...

A 24-year-old woman attends the A&E in an agitated state. She complains of being anxious and has insomnia. On examination, she has tachycardia and tremor. — Choose...

Choose...
Opiate withdrawal
Cocaine withdrawal
Benzodiazepine withdrawal
LSD intoxication
Cocaine intoxication
Opiate intoxication
Cannabis intoxication
Cannabis withdrawal
Amphetamine intoxication
Caffeine withdrawal

Opiate withdrawal effects: Morphine and heroin withdrawal syndrome begins 6 to 8 hours after the last dose, peaks in 2 days and reduces in a week usually. The clinical features include dysphoric mood, nausea or vomiting, muscle aches, lacrimation or rhinorrhea, pupillary dilation, piloerection (goose flesh), or sweating, diarrhea, yawning, fever and insomnia.

Amphetamine Intoxication effects: Tachycardia or bradycardia - arrhythmias may be fatal, pupillary dilation, elevated or lowered blood pressure, perspiration or chills, nausea or vomiting, psychomotor agitation or retardation, muscular weakness, respiratory depression, chest pain, or cardiac arrhythmias. Psychological effects such as euphoria, changes in sociability, anxiety, tension, stereotyped behaviours, impaired judgment, etc. The hallmark of amphetamine-induced psychosis is the paranoia. Sensitisation is suggested to be the biological mechanism behind stimulant-induced psychosis.

Benzodiazepine withdrawal: Autonomic hyperactivity, increased tremor, insomnia, nausea or vomiting, transient visual, tactile, or auditory hallucinations or illusions, psychomotor agitation, anxiety (most prominent), grand mal seizures and kinaesthetic hallucinations reported in a few.

Excerpt from Amphetamine Related Psychiatric Disorders: Clinical Presentation. (n.d.). Retrieved from http://emedicine.medscape.com/article/289973-clinical.

The correct answer is: A 25-year-old woman who is on a controlled drug prescription went on a short holiday for three days and forgot to take her prescribed medication. When she returned home, she developed diarrhoea, running nose and goose flesh. On examination, she had dilated pupils and tachycardia. → Opiate withdrawal, A 30-year-old gentleman presented to the A&E with paranoid delusions and prominent visual hallucinations of very rapid onset following ingesting of a street drug. On examination, he had tachycardia, hypertension and pupillary dilatation. → Amphetamine intoxication, A 24-year-old woman attends the A&E in an agitated state. She complains of being anxious and has insomnia. On examination, she has tachycardia and tremor. → Benzodiazepine withdrawal

Question **42**

Not answered

Marked out of 6.00

HiY Assessment CXEMI093

ICD diagnostic features

For each of the following ICD-11 diagnoses given below, choose the most common clinical presentation from the list

1. Mania (TWO)

2. Delirium (TWO)

3. Delusional disorder (TWO)

Transient delusion
Systematized delusion
Bizarre delusion
Persecutory delusion
Loss of consciousness
Affective changes
Auditory hallucinations
Visual hallucinations

Explanation:

Mania: Persecutory delusions are common in psychotic mania next only to grandiose delusions. Though visual hallucinations of a religious nature are commoner in mania than schizophrenia, these are still uncommon as a diagnostic feature. Delusions accompanying mania are often short-lived (but not as transient as in delirium) and are poorly systematized. Affective changes are often seen in mania.

Delirium: Visual hallucinations clearly outnumber auditory hallucinations in delirium. Transient delusions are often seen in delirium.

Delusional disorder: Most delusional disorders often have a single delusion and usually these are well systematised, and are often persecutory in theme. In delusional disorders, usually there are no hallucinations (though olfactory and tactile hallucinations may be present as secondary phenomenon); psychosocial functioning may be at a higher level than expected for schizophrenia.

The correct answer is:

Mania (TWO)- Persecutory delusion, Affective changes

Delirium (TWO)- Transient delusion, Visual hallucinations

Delusional disorder (TWO)- Systematized delusion, Persecutory delusion

Question **43**

Not answered

Marked out of 3.00

Culture-bound syndromes

Identify the culture-bound syndromes using the descriptions given below:

A West-African student complains of being too tired to do his university work and is struggling to concentrate. Choose...

A young Indonesian gentleman has an excessive fear of dying as a result of his genital retracting into his abdomen. Choose...

A Chinese gentleman displays an excessive fear of coldness. Choose...

Choose...
Pa-Leng
Windigo
Brain Fag
Latah
Piblokto
Koro
Amok
Dhat

Explanation:

Brain fag is described particularly in Western African students. It is characterised by distress from 'thinking too much', reduced concentration, poor memory, blurred vision, and head/neck pain (often described as tightness, pressure, heat, or burning). Symptoms closely resemble anxiety, depressive, or somatoform disorders.

Koro is a form of 'genital retraction syndrome' – the fear or delusion that the genitals are retracting into the abdomen and that death will occur once this has happened. It occurs predominantly in young, single males in Asia and the Middle East, with epidemics described in the Malay Archipelago, Thailand, China, India, Singapore, and Israel. Sporadic cases have been reported in Africa, Europe, and North America.

Pa-leng is a culture-bound syndrome found in China and southeast Asia, characterised by pathological fear of cold (frigophobia) and of wind (anemophobia), believed to produce fatigue, impotence, and death.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 990-991.

Colman AM. Oxford Dictionary of Psychology, 4 th Edition. 2015. p. 542.

The correct answer is: A West-African student complains of being too tired to do his university work and is struggling to concentrate. → Brain Fag, A young Indonesian gentleman has an excessive fear of dying as a result of his genital retracting into his abdomen. → Koro, A Chinese gentleman displays an excessive fear of coldness. → Pa-Leng

Question **44**

Not answered

Marked out of 3.00

Choose...
Amok
Zar
Susto
Dhat
Frigophobia
Latah
Koro
Brain Fag
Windigo
Taijin Kyofusho

Diagnosing culture-bound syndromes

What is the most appropriate diagnosis in the following cases?

A 25-year-old South East Asian man who was depressed for a while who suddenly becomes indiscriminately violent. Choose...

A 27-year-old Native American man who is convinced that he has become a cannibal. Choose...

A 29-year-old South American man who has become acutely anxious as he has "lost his soul". Choose...

Explanation:

Amok is a dissociative episode characterised by a period of depression followed by an outburst of violent, aggressive, or homicidal behaviour. Episodes tend to be caused by a perceived insult and are often accompanied by persecutory ideas, automation, amnesia, and exhaustion. Patients return to premorbid states following the episode. Amok seems to be prevalent only among males. The term originated in Malaysia.

Witiko psychosis (or Windigo) refers to a person developing the delusion of transforming into a cannibalistic monster and craving human flesh under the threat of starvation in severely cold weather. (Note: The term 'Witiko' is more appropriate to capture this condition, and the Western interpretation of what is described as Windigo being attributed to the ethnic groups of Cree and Metis people has been disputed.)

Susto is a culture-related syndrome found in Peru and other parts of Latin America and among other Spanish-speaking communities. Following a frightening experience, the soul is believed to have departed the body, resulting in symptoms such as psychomotor agitation, anorexia, insomnia or hypersomnia, nightmares, and depressed mood, together with somatic complaints such as muscle and joint pain, headaches, and diarrhoea.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 1053.

Tseng WS. From peculiar psychiatric disorders through culture-bound syndromes to culture-related specific syndromes. Transcultural psychiatry. 2006 Dec;43(4):554-76.

Carlson ND. Reviving Witiko (Windigo): An ethnohistory of “cannibal monsters” in the Athabasca district of northern Alberta, 1878–1910. Ethnohistory. 2009;56(3):355-94.

Colman AM. Oxford Dictionary of Psychology, 4th Edition. 2015. p. 746.

The correct answer is: A 25-year-old South East Asian man who was depressed for a while who suddenly becomes indiscriminately violent. → Amok, A 27-year-old Native American man who is convinced that he has become a cannibal. → Windigo, A 29-year-old South American man who has become acutely anxious as he has "lost his soul". → Susto

Finish review

PREVIOUS ACTIVITY
EMIs Assessment Part 1

Jump to...

Drop here!

© 2022 SPMM Course Limited, UK.

Refund Policy | Terms & Conditions